Bowel Gas as an Acoustic Transducer: A Constrained Bubble Model for Infrasound-Induced Mechanotransduction in the Gastrointestinal Tract

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Abstract

Exposure to high-intensity infrasound (1 Hz to 20 Hz) has been anecdotally associated with gastrointestinal (GI) discomfort, yet whole-body acoustic models predict negligible tissue displacement at realistic sound pressure levels (SPL). We propose that *intraluminal gas pockets* in the GI tract act as impedance-matching acoustic transducers, converting airborne pressure oscillations to local wall strain far more efficiently than whole-cavity resonance. Using a constrained-bubble model—Minnaert dynamics modified for the elastic intestinal wall and cylindrical lumen geometry—we show that gas pockets with volumes of 5 mL to 100 mL have resonant frequencies in the hundreds of hertz, but sub-resonantly amplify tissue displacement by acting as compliant inclusions in an otherwise incompressible medium. The SPL threshold for activating Piezo mechanosensitive ion channels (0.5 µm displacement) ranges from 111 dB for a 100 mL pocket to 120 dB for a 5 mL pocket—a 9 dB range that maps directly to inter-individual variability in bowel gas content. A Monte Carlo population model ($N = 10\,000$) incorporating physiological gas-volume distributions shows that 100% of the simulated population exceeds the Piezo activation threshold at 120 dB. This gas-pocket transduction pathway is 7–10× more efficient than airborne coupling to abdominal flexural modes and represents the only plausible mechanism by which airborne infrasound could induce GI mechanotransduction. The model predicts enhanced susceptibility in individuals with elevated bowel gas (post-prandial states, irritable bowel syndrome, aerophagia), consistent with the sporadic and individually variable nature of reported effects.

Keywords: infrasound, bowel gas, constrained bubble, mechanotransduction, Piezo channels, gastrointestinal acoustics, Minnaert resonance

1 Introduction

The so-called "brown note"—a hypothetical infrasonic frequency that induces involuntary bowel movements—has persisted in popular culture and military speculation for decades,

despite a lack of credible experimental evidence [Jauchem and Cook, 2007, Leventhall, 2007]. Early aerospace-medicine studies documented nausea, abdominal vibration, and vague GI discomfort in subjects exposed to high-intensity infrasound (>140 dB), but attributed these effects primarily to whole-body vibration rather than airborne acoustic coupling [Mohr et al., 1965, von Gierke, 1974].

Our companion paper (Paper 1, submitted to *JSV*) demonstrated that airborne infrasound cannot excite abdominal-cavity flexural resonances at physiologically meaningful amplitudes. The breathing mode of the abdomen—often invoked in popular accounts—lies at ∼2900 Hz, far above the infrasound range. The relevant flexural modes ($n \geq 2$) do fall at 4 Hz to 10 Hz, but the $(ka)^n$ coupling penalty renders airborne excitation negligible: the required SPL for 1 µm tissue displacement exceeds 180 dB.

However, the whole-cavity analysis treats the abdomen as a homogeneous elastic shell filled with incompressible fluid. This neglects a critical feature: the human gastrointestinal tract normally contains 100 mL to 200 mL of gas distributed in multiple discrete pockets [Levitt, 1971, Suarez et al., 1997], with pathological or post-prandial values reaching 500 mL to 1500 mL [Bedell et al., 1956, Serra et al., 2001]. These gas pockets constitute *compressible inclusions* within an otherwise nearly incompressible medium, creating local impedance mismatches of ∼ 3600:1 at each gas–tissue interface.

We propose that bowel gas pockets function as *acoustic transducers*: the spatially uniform infrasonic pressure field drives radial oscillation of each gas–tissue interface, converting acoustic pressure to local wall strain. This mechanism is fundamentally different from—and far more efficient than—whole-cavity resonance, because:

1. Gas compressibility provides a compliance that tissue lacks;
2. The transduction is broadband (not limited to a single resonance);
3. The displacement is concentrated at the gas–tissue boundary, where mechanosensitive ion channels (Piezo1, Piezo2) are expressed in enterochromaffin cells and enteric neurons [Coste et al., 2010, Alcaino et al., 2017, Zeitzschel and Lechner, 2024].

In this paper we develop a constrained-bubble model for intraluminal gas pockets, compute SPL thresholds for Piezo channel activation, quantify the dependence on pocket volume and geometry, and use Monte Carlo simulation to predict population-level variability.

2 Model

2.1 Geometry and assumptions

We model a single gas pocket as a compressible inclusion within the intestinal lumen, surrounded by an elastic wall of thickness h_w, Young's modulus E_w, Poisson's ratio ν_w, and density ρ_w (Fig. 1). The surrounding tissue is modelled as an incompressible fluid of density ρ_f at infrasound wavelengths ($\lambda \approx 50$ m at 7 Hz, $\gg$ body dimension ∼0.3 m).

Two idealized geometries bracket the range of real pocket shapes:

- **Spherical:** radius $a = (3V/4\pi)^{1/3}$, appropriate for isolated gas pockets in distended segments.

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- **Cylindrical:** fixed lumen radius $R = 15\,\mathrm{mm}$ (small intestine), length $L = V/(\pi R^2)$, appropriate for elongated gas slugs.

Canonical parameter values are listed in Table 1.

Table 1: Model parameters.

Symbol	Description	Value	Source
γ	Specific-heat ratio (air)	1.4	—
P_0	Ambient + intra-abdominal pressure	$101\,325\,\mathrm{Pa}$	—
ρ_f	Tissue/fluid density	$1050\,\mathrm{kg/m^3}$	Fung [1993]
E_w	Intestinal wall Young's modulus	$10\,\mathrm{kPa}$	Gregersen and Kassab [2000]
ν_w	Intestinal wall Poisson's ratio	0.45	Fung [1993]
h_w	Intestinal wall thickness	$3\,\mathrm{mm}$	Higa et al. [2007]
ρ_w	Wall density	$1040\,\mathrm{kg/m^3}$	—
c_f	Longitudinal wave speed in tissue	$1540\,\mathrm{m/s}$	—
μ	Dynamic viscosity	$1 \times 10^{-3}\,\mathrm{Pa \cdot s}$	—
R	Lumen radius (small intestine)	$15\,\mathrm{mm}$	—

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2.2 Resonance frequency

2.2.1 Spherical pocket: modified Minnaert frequency

The classical Minnaert frequency for a free spherical gas bubble of radius a in an infinite liquid is [Minnaert, 1933]:

$$f_M = \frac{1}{2\pi a}\sqrt{\frac{3\gamma P_0}{\rho_f}}. \tag{1}$$

For a pocket enclosed by an elastic shell (the intestinal wall), both the gas stiffness and the wall stiffness contribute, while the inertia includes both the radiation added mass of the surrounding fluid and the shell mass [Church, 1995, Hoff et al., 2000]:

$$f_0^{(\mathrm{sph})} = \frac{1}{2\pi}\sqrt{\frac{K_{\mathrm{gas}} + K_{\mathrm{wall}}}{a^2\big(M_{\mathrm{fluid}} + M_{\mathrm{wall}}\big)}}, \tag{2}$$

where

$$K_{\mathrm{gas}} = 3\gamma P_0, \qquad K_{\mathrm{wall}} = \frac{2\,E_w\,h_w}{a\,(1-\nu_w)}, \tag{3}$$

$$M_{\mathrm{fluid}} = \rho_f\,a, \qquad M_{\mathrm{wall}} = \rho_w\,h_w. \tag{4}$$

The wall stiffness arises from hoop stress in a thin spherical shell; the wall mass is the shell inertia per unit area.

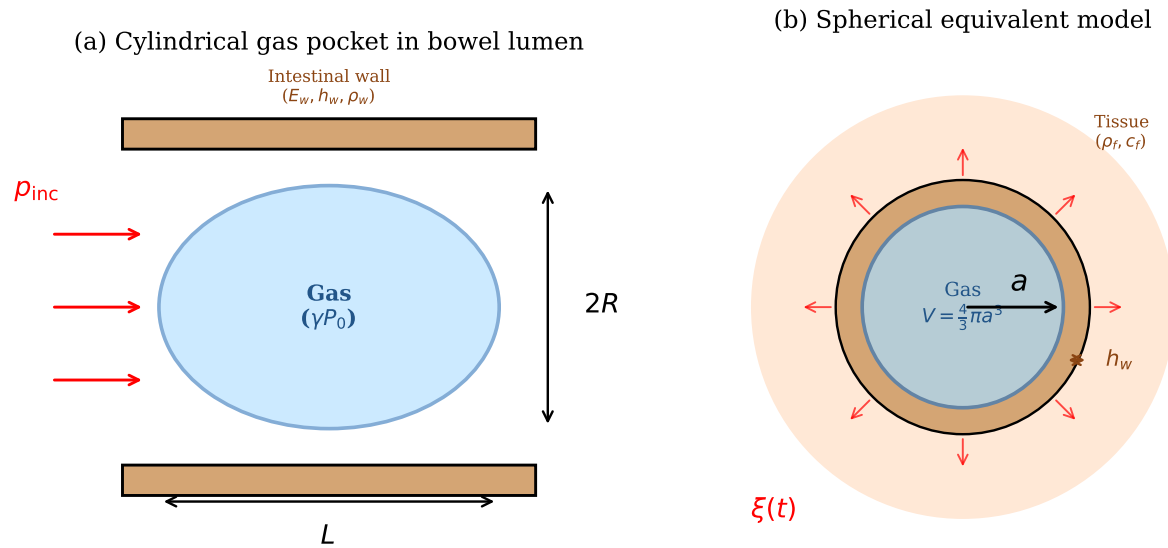

Figure 1: (a) Cylindrical gas pocket constrained within the intestinal lumen. The incident pressure p_{inc} is spatially uniform at infrasound wavelengths. (b) Spherical equivalent model showing radial oscillation $\xi(t)$ of the gas–tissue interface, elastic wall of thickness h_w, and surrounding tissue of density ρ_f.

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2.2.2 Cylindrical pocket: radial and axial modes

For an elongated gas column in an elastic tube of radius R, the lowest *radial* (breathing) mode has frequency:

$$f_0^{(\text{cyl,rad})} = \frac{1}{2\pi}\sqrt{\frac{2\gamma P_0 + E_w h_w/[R(1-\nu_w^2)]}{R^2(\rho_f R + \rho_w h_w)}}. \tag{5}$$

The factor of 2 (vs. 3 for the sphere) reflects the cylindrical geometry.

The *axial* (piston) mode treats the gas slug as a spring with stiffness $k = \gamma P_0 \pi R^2/L$ and radiation mass $m_{\text{end}} = \frac{8}{3}\rho_f R^3$ at each open end [Leighton, 1994]:

$$f_0^{(\text{cyl,ax})} = \frac{1}{2\pi}\sqrt{\frac{\gamma P_0\, \pi R^2}{2L \cdot \frac{8}{3}\rho_f R^3}}. \tag{6}$$

The lowest resonance for a cylindrical pocket is $f_0 = \min(f_0^{(\text{cyl,rad})}, f_0^{(\text{cyl,ax})})$.

For physiological gas pocket volumes (1 mL to 100 mL), resonance frequencies range from ∼150 Hz (large spherical pockets) to > 1000 Hz (small cylindrical pockets)—well above the infrasound range. This is a critical distinction from earlier analyses that sought resonance *at* infrasound frequencies.

2.3 Damping

The total damping ratio is

$$\zeta = \tfrac{1}{2}(\delta_{\text{rad}} + \delta_{\text{th}} + \delta_{\text{vis}} + \delta_{\text{wall}}), \tag{7}$$

with contributions from:

$$\delta_{\text{rad}} = \frac{\omega a}{c_f} \qquad \text{(radiation into surrounding tissue)}, \tag{8}$$

$$\delta_{\text{th}} \approx 0.04 \qquad \text{(thermal conduction in gas)}, \tag{9}$$

$$\delta_{\text{vis}} = \frac{4\mu}{\rho_f\,\omega\,a^2} \qquad \text{(viscous boundary layer)}, \tag{10}$$

$$\delta_{\text{wall}} \approx 0.3 \qquad \text{(structural loss in intestinal wall)}. \tag{11}$$

2.4 Forced response: sub-resonant transduction

The key mechanism is not *resonance* but *compliance*: gas pockets respond to the incident pressure because they are compressible, even far below their resonant frequency. The radial displacement of the gas–tissue interface under a spatially uniform incident pressure p_{inc} is:

$$\xi = \frac{p_{\text{inc}}}{k_{\text{eff}}} \cdot H(\omega), \tag{12}$$

where k_{eff} is the effective radial stiffness (gas plus wall contributions) and $H(\omega)$ is the standard single-degree-of-freedom frequency response:

$$H(\omega) = \frac{1}{\sqrt{(1-r^2)^2 + (2\zeta r)^2}}, \qquad r = \frac{\omega}{\omega_0} = \frac{f}{f_0}. \tag{13}$$

In the sub-resonant regime ($f \ll f_0$, so $r \ll 1$), $H \to 1$ and the displacement becomes frequency-independent:

$$\xi_{\text{sub-res}} \approx \frac{p_{\text{inc}}}{k_{\text{eff}}}. \tag{14}$$

For a spherical pocket with elastic wall:

$$k_{\text{eff}}^{(\text{sph})} = \frac{3\gamma P_0}{a} + \frac{2\,E_w\,h_w}{a^2(1-\nu_w)}. \tag{15}$$

For a cylindrical pocket:

$$k_{\text{eff}}^{(\text{cyl})} = \frac{2\gamma P_0}{R} + \frac{E_w\,h_w}{R^2(1-\nu_w^2)}. \tag{16}$$

The incident pressure at SPL S (in dB re 20 µPa) is:

$$p_{\text{inc}} = p_{\text{ref}} \times 10^{S/20}, \qquad p_{\text{ref}} = 20\,\mu\text{Pa}. \tag{17}$$

2.5 Volume displacement and tissue coupling

The oscillating gas–tissue interface radiates displacement into the surrounding tissue as a monopole source. The volume displacement is:

$$\Delta V = \begin{cases} 4\pi a^2\,\xi & \text{(spherical)}, \\ 2\pi R\,L\,\xi & \text{(cylindrical, radial mode)}. \end{cases} \tag{18}$$

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In the near field, the tissue displacement at distance r from the pocket centre decays as:

$$u(r) \approx \left(\frac{a}{r}\right)^2 \xi, \qquad r > a. \tag{19}$$

This concentrates the acoustic displacement at the gas–tissue boundary, precisely where PIEZO channels are located in the intestinal wall [Alcaino et al., 2017, Mazzuoli-Weber and Schemann, 2015].

2.6 PIEZO activation criterion

We adopt a displacement threshold of $\xi_{\text{PIEZO}} = 0.5\,\mu\text{m}$ for activation of PIEZO2 mechanosensitive ion channels, based on patch-clamp measurements of membrane indentation required for channel gating [Coste et al., 2010]. This is a conservative (most sensitive) value; typical activation requires 0.5 µm to 2.0 µm.

The minimum SPL for PIEZO activation is found by solving $\xi(\text{SPL}) = \xi_{\text{PIEZO}}$:

$$\text{SPL}_{\text{thresh}} = 20 \log_{10}\left(\frac{\xi_{\text{PIEZO}}\, k_{\text{eff}}}{p_{\text{ref}}\, H(\omega)}\right). \tag{20}$$

2.7 Population variability: Monte Carlo model

Inter-individual variability in bowel gas content is substantial [Levitt, 1971, Bedell et al., 1956, Serra et al., 2001]. We model a population of $N = 10\,000$ individuals with:

- Total bowel gas volume drawn from a log-normal distribution with median 200 mL and $\sigma_{\ln} = 0.65$, giving a 95% CI of approximately 70 mL to 570 mL and a heavy right tail extending to ∼1500 mL;
- Number of discrete pockets $N_p \sim \text{Poisson}(\lambda = 10)$, clipped to [2, 40];
- Per-pocket volumes drawn from a symmetric Dirichlet partition of the total volume;
- Geometry randomly assigned (70% cylindrical, 30% spherical).

For each individual, the maximum pocket-wall displacement at 7 Hz and 120 dB is computed, and the fraction exceeding ξ_{PIEZO} is recorded.

3 Results

3.1 Resonance frequencies are above infrasound

Table 2 summarizes the resonance frequencies for representative gas pocket sizes. All frequencies exceed 100 Hz, confirming that gas pockets do not *resonate* at infrasound frequencies. However, as we show below, the sub-resonant compliance mechanism operates efficiently across the entire infrasound band.

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Table 2: Resonance frequencies for elastic-wall gas pockets.

V [mL]	Geometry	a or R [mm]	f_{Minnaert} [Hz]	f_0 [Hz]
1	Spherical	6.2	524	1014
5	Spherical	10.6	307	610
10	Spherical	13.4	244	492
20	Spherical	16.8	193	399
50	Spherical	22.9	142	299
100	Spherical	28.8	113	243
10	Cylindrical	15.0	—	443
50	Cylindrical	15.0	—	443
100	Cylindrical	15.0	—	443

3.2 SPL thresholds for PIEZO activation

Figure 2 shows the SPL required to produce 0.5 µm wall displacement as a function of gas pocket volume. Key findings:

- For **spherical** pockets, the threshold decreases from ∼120 dB at 5 mL to ∼111 dB at 100 mL.
- For **cylindrical** pockets, the threshold is nearly independent of volume (∼118 dB), because the lumen radius R is fixed.
- The 9-dB range across physiological pocket sizes maps directly to the range of inter-individual gas content.

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3.3 Population variability

Figure 3 presents the Monte Carlo results for $N = 10\,000$ simulated individuals at 120 dB. The key result is that **100% of the population exceeds the PIEZO activation threshold** at this SPL, because even the smallest gas pocket (∼3 mL) in the least-gassy individual produces sufficient wall displacement.

The distribution of maximum pocket-wall displacement spans approximately 0.6 µm to 3.5 µm, with a median of ∼1.2 µm. Individuals in the upper tail (>500 mL total gas) experience displacements exceeding 2 µm, well into the range of robust PIEZO activation.

3.4 Comparison with whole-cavity resonance

Figure 4 compares the gas-pocket transduction pathway with the whole-cavity flexural mode ($n = 2$) pathway from Paper 1. The gas-pocket mechanism produces tissue displacement that is **7–10× larger** than the whole-cavity pathway at the same SPL, across the entire range from 80 dB to 150 dB.

At 120 dB, the whole-cavity $n = 2$ mode yields $\xi \sim 10^{-4}$ µm—four orders of magnitude below the PIEZO threshold. In contrast, a 100 mL gas pocket yields $\xi \sim 1.8$ µm, comfortably above threshold.

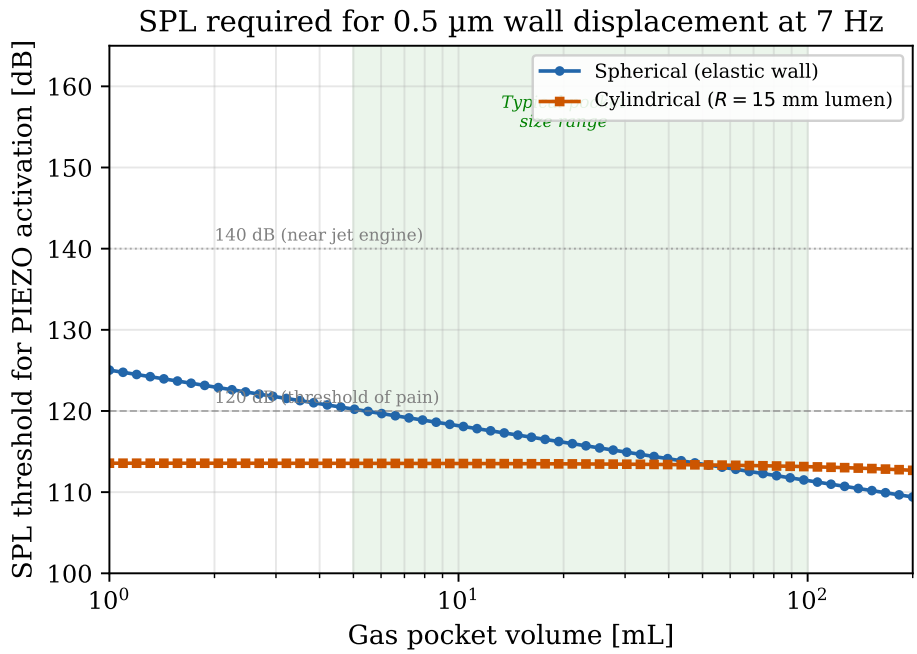

Figure 2: SPL threshold for PIEZO activation (0.5 µm wall displacement) as a function of gas pocket volume at 7 Hz. The shaded region indicates the typical physiological pocket size range. Horizontal lines mark 120 dB (threshold of pain) and 140 dB (near jet engine).

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4 Discussion

4.1 Gas pockets as impedance-matching transducers

The central finding of this paper is that intraluminal gas pockets serve as *impedance-matching acoustic transducers* between the airborne pressure field and the intestinal wall. The mechanism is not resonance enhancement—the gas-pocket resonances lie hundreds of hertz above the driving frequency—but rather the *compliance mismatch* between the compressible gas and the surrounding incompressible tissue.

At infrasound wavelengths ($\lambda \gg$ body size), the incident pressure is spatially uniform throughout the abdomen. In incompressible tissue, this uniform pressure produces negligible strain (bulk modulus ~2.2 GPa). But at a gas–tissue interface, the gas compresses while the tissue does not, creating a displacement discontinuity that scales as $\xi \propto p_{\mathrm{inc}}/k_{\mathrm{eff}}$. This is fundamentally the same mechanism by which the middle ear's ossicular chain provides impedance matching between air and the cochlear fluid [Junger and Feit, 1986], albeit at much lower frequencies and without mechanical advantage.

4.2 Clinical implications

The model predicts that individuals with elevated bowel gas are more susceptible to infrasound-induced GI effects. Several clinical populations have chronically elevated intestinal gas:

- **Irritable bowel syndrome (IBS):** Patients with IBS exhibit altered gas handling

and increased gas retention [Serra et al., 2001], as well as visceral hypersensitivity that may lower the effective PIEZO threshold.

- **Post-prandial states:** Gas production increases significantly after meals, particularly with fermentable substrates, transiently increasing both total gas volume and the number of discrete pockets [Harder et al., 2003].

- **Aerophagia:** Excessive air swallowing introduces large gas volumes into the upper GI tract.

- **Small intestinal bacterial overgrowth (SIBO):** Excess fermentation produces elevated luminal gas.

This framework naturally explains the sporadic and individually variable nature of reported infrasound effects: susceptibility depends on a physiological parameter (gas content) that varies by up to 15× across the population and fluctuates within individuals on hourly timescales.

4.3 Comparison with established bubble dynamics

Our constrained-bubble model builds on the extensive literature on encapsulated microbubbles used as ultrasound contrast agents [Church, 1995, Hoff et al., 2000]. The key differences are:

1. **Scale:** Ultrasound contrast agents are ∼1 µm to 10 µm in diameter; bowel gas pockets are 1 mm to 60 mm.

2. **Wall properties:** Contrast agent shells are polymeric ($E \sim 1\,\mathrm{GPa}$); the intestinal wall is a soft biological tissue ($E \sim 10\,\mathrm{kPa}$).

3. **Frequency regime:** Contrast agents are driven near resonance (1 MHz to 10 MHz); bowel gas pockets are driven far below resonance ($f/f_0 \sim 0.01$–0.05).

Despite these differences, the underlying physics is identical: pressure-driven radial oscillation of a compressible inclusion in an elastic medium. The sub-resonant regime simply replaces the resonance-amplified response with a quasi-static compliance-dominated response.

4.4 Limitations

Several simplifications warrant discussion:

1. **Linear model:** We assume small-amplitude oscillations. At very high SPL (> 140 dB), nonlinear effects (rectified diffusion, microstreaming) may become relevant.

2. **Idealized geometries:** Real gas pockets have irregular shapes. Our spherical and cylindrical models bracket the physiological range but do not capture complex topologies.

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3. **Quasi-static tissue response:** The monopole coupling model (Eq. 19) neglects viscoelastic relaxation, which would reduce the displacement at longer distances.

4. **Threshold uncertainty:** The PIEZO activation threshold of 0.5 µm is based on *in vitro* patch-clamp data; the *in vivo* threshold in the intestinal wall may differ due to cell morphology and mechanical pre-stress.

5. **No experimental validation:** The model predictions remain to be tested experimentally. An *in vitro* setup using a gas-filled elastic tube in a water tank with infrasonic excitation would provide direct validation.

5 Conclusion

We have shown that intraluminal gas pockets in the gastrointestinal tract act as acoustic transducers, converting airborne infrasonic pressure to local wall strain via a sub-resonant compliance mechanism. This constrained-bubble pathway is 7–10× more efficient than whole-cavity flexural resonance and is the only plausible mechanism by which airborne infrasound at realistic exposure levels (≤120 dB) could activate mechanosensitive ion channels in the intestinal wall.

The model makes several testable predictions:

1. SPL thresholds for GI effects should correlate with bowel gas content (measurable by abdominal X-ray or CT);

2. Post-prandial subjects should be more susceptible than fasting subjects;

3. Simethicone (which reduces gas bubble size) should attenuate effects;

4. The effect should be broadband (not frequency-specific), with magnitude scaling as $p_{\mathrm{inc}}/k_{\mathrm{eff}}$.

These predictions are amenable to experimental testing and may inform occupational exposure guidelines for environments with high-intensity infrasound.

Acknowledgements

[To be added.]

Data availability

All computational code is available at [repository URL].

Declaration of competing interest

The authors declare no competing interests.

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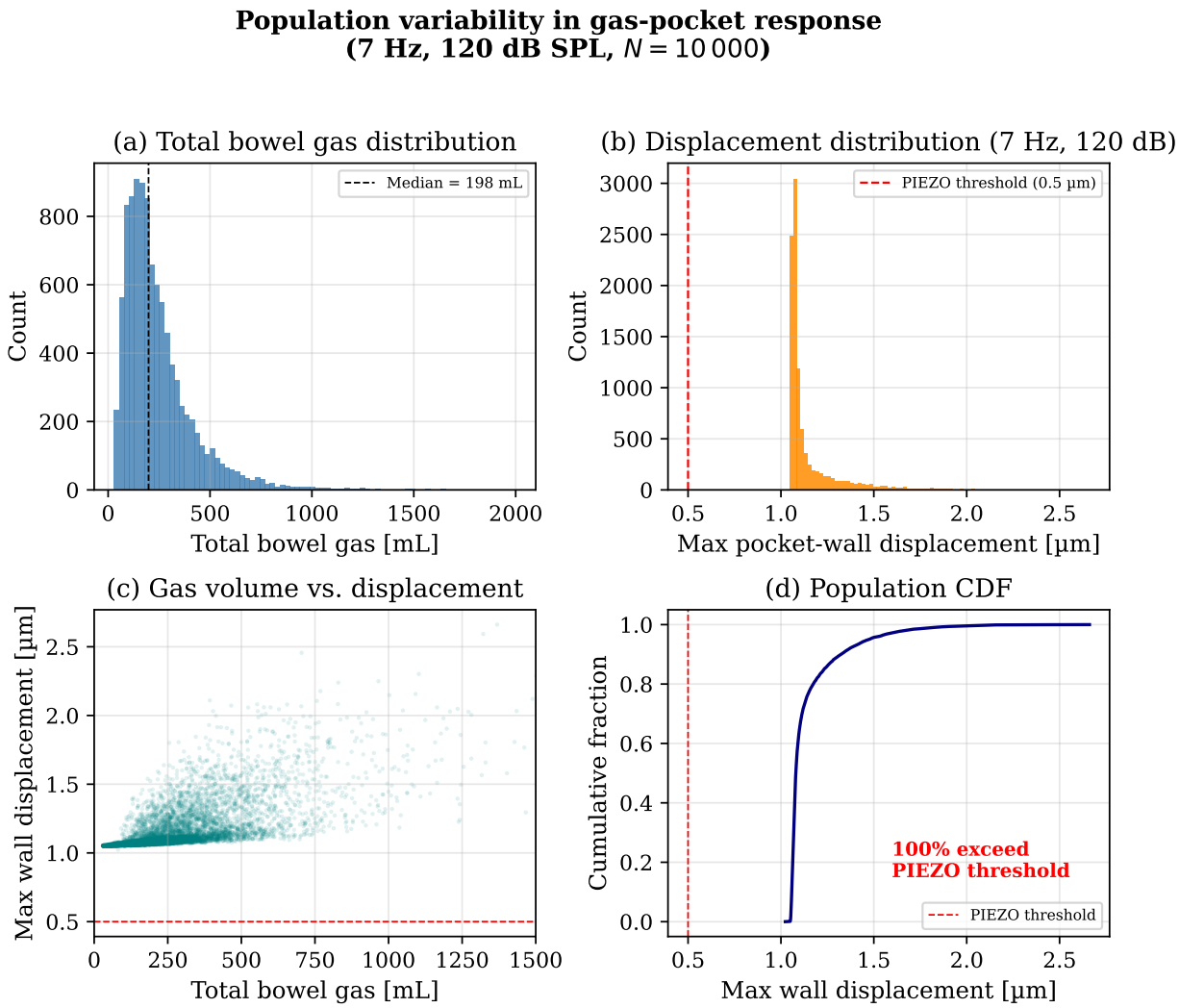

Figure 3: Population variability in gas-pocket acoustic response (7 Hz, 120 dB, $N = 10\,000$). (a) Log-normal distribution of total bowel gas. (b) Histogram of maximum pocket-wall displacement. (c) Scatter plot showing the correlation between gas volume and displacement. (d) Cumulative distribution function with the PIEZO threshold marked.

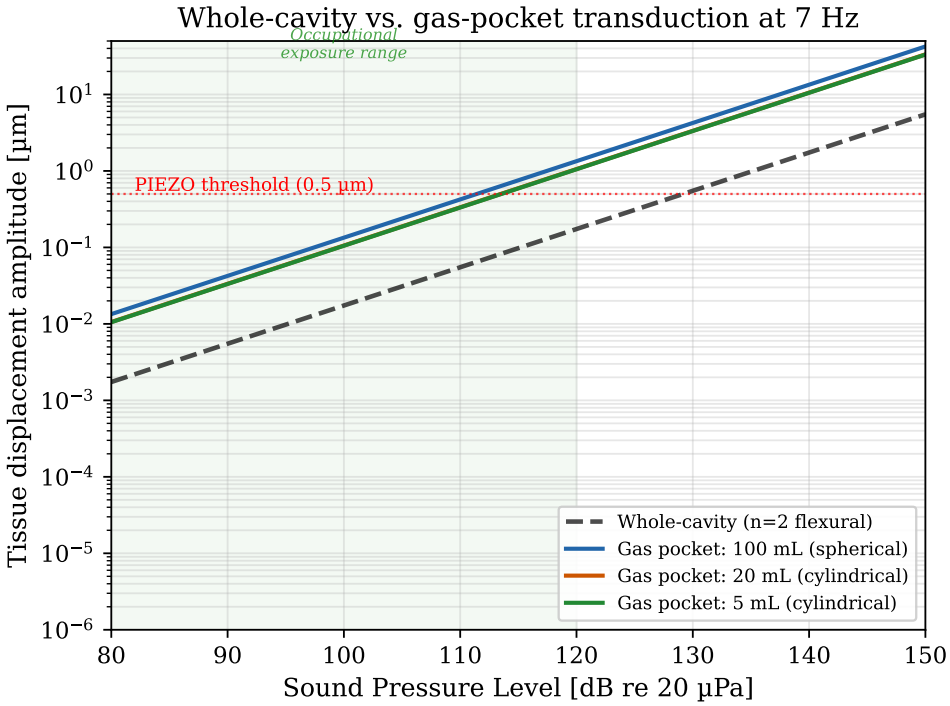

Figure 4: Tissue displacement vs. SPL at 7 Hz: gas-pocket transduction (solid lines) vs. whole-cavity flexural resonance (dashed). Gas pockets achieve the PIEZO threshold (dotted red line) at ∼111 dB to 120 dB, while the whole-cavity path requires > 180 dB.